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Name: عبدالله سيد ابراهيم حسين National ID: 26305141400553	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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A patient of lung cancer for Pemetrexed with platinum-based chemotherapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١١-٤٦٢١٥٨٦

DMS

تدعيم
الورق عن السيد السيد DMS



Medical Report

Patient Name :	عبدالله سيد ابراهيم حسين	Age :	63 Y
Sex :	☐ Male ☐ Female	Comorbidity :	HTN, DM
ID No. :	12494	Telephone :	
Family History :		Marital Status :	☒ Married ☐ Divorced ☐ Single
Menopausal Status :		Reason of MDT :	
		MDT name::	Thoracic cancers

Medical Report

Diagnosis :	
Notes :	
History :	60 years old heavy smoker for 45 yrs 9/24 pet scan showed rt ULL Pulm nodule 2 cm suv 14 HPE adenoca grade 2 patient underwent surgery PT1AN1 patient refused adj chemo now pet scan on 27/6 /26 showed few hypermetabolic rt sided nodular lung lesion with irregular pleural thickening , multiple supra infra diaphragmatic lns and HFL LF ADRENAL
Investigations:	PDL-1 <1%, EGFR, ALK wild. For NGS at CDC Pt already started chemotherapy 2 weeks ago Started Alimta / Carboplatin on 8/7/2026
Recommendations:	To continue Alimta / Carboplatin
MDT DECISION:	To Continue Alimta- Carboplatin with dose adjustment according to toxicity

Specialty	Surgery	Pathology	Radiology
Medical oncology			

Name: عبدالله سيد ابراهيم حسين		GUSTAVE ROUSSY	
MR. : 12494			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 60-year-old heavy smoker with a 45-year smoking history. A PET/CT performed on 24/07/2026 revealed a right upper lobe pulmonary nodule measuring 2 cm with an SUV of 14. Histopathology confirmed grade 2 adenocarcinoma. The patient underwent surgical resection with pathology staging of PT1AN1. The patient declined adjuvant chemotherapy. A subsequent PET/CT on 27/06/2026 demonstrated several hypermetabolic right-sided pulmonary lesions with irregular pleural thickening, multiple supra- and infra-diaphragmatic lymph nodes, and left adrenal involvement. EGFR, ALK, and PD-L1 testing were negative (PD-L1 < 1%, EGFR wild-type, ALK wild-type). NGS was ordered at CDC. The patient began systemic therapy two weeks prior to the 20/07/2026 note, receiving Alimta (pemetrexed) and carboplatin (C5 AUC 5) on 08/07/2026. The regimen was initiated in preparation for multidisciplinary team (MDT) discussion. On 03/08/2026 the MDT recommended continuation of Alimta and carboplatin with dose adjustments based on toxicity. The patient reported neutropenic fever, oral mucositis (grade I), gastrointestinal symptoms (grade II), and diarrhea (grade II) during cycle 2 of Alimta and carboplatin. Supportive care included granulocyte colony-stimulating factor (G-CSF) and supplemental therapy. Laboratory values were acceptable, with weight 87 kg and height 175 cm. On 24/08/2026 the patient proceeded to cycle 3 of Alimta and carboplatin. Laboratory parameters remained within acceptable limits, and the patient received a second prophylactic dose of Neupogen.			
تحريراً في ,			
		التاريخ : 20-09-2026	الوقت :
		حنان عزت شفيق تاودروس	الطبيب المعالج :
يعتمد ,			

أ.د. أحمد جمال الحدرى
أ.د. محمد أبو الهدي درويش
أ.د. عمر حسنين عمر
أ.د. وحيد حسين طنطاوى
أ.د. ياسر عبد العظيم عباس
أ.د. أحمد محمد مزيب
أستاذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MR. ABDALLAH SAYED IBRAHIM
PROF. DR. HANY HASSAN

PATIENT ID: 1516881
STUDY DATE: 27/06/2026

F18- FDG PET/CT EXAMINATION

History:-

64 years old male patient with history of right upper lobectomy for adenocarcinoma 2 years ago, now presented with right chest pain with right pleural effusion, referred for PET/CT examination.

Procedure:-

- 7.6 mCi 18F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 149 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- A diagnostic 128 MDCT post contrast examination was taken after I.V. non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:-

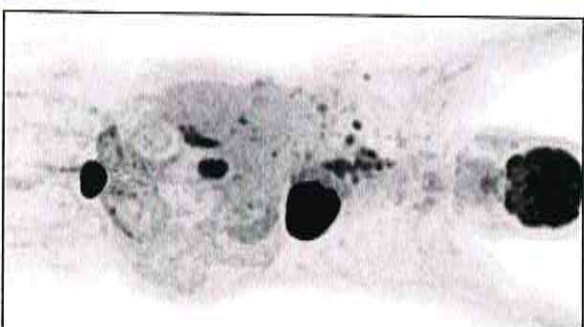
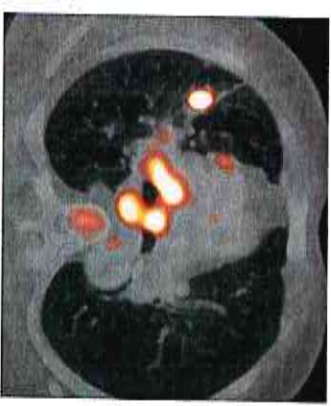
No available previous PET/CT studies, yet the current study revealed:

A) Head and Neck:-

- The neck is devoid of any abnormally hypermetabolic lymph nodes.
- The brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- No parenchymal thyroid focal lesions could be elicited.

B) Chest:-

- Evidence of right upper lobectomy with hyperdense surgical clips seen related to the origin of the right upper lobe bronchus. An ill-defined hypermetabolic nodular lesion is seen related to the upper aspect of the right middle lobe with related thick fibrotic band, measuring $\pm 19 \times 15$ mm and achieves up to 9.14 SUVmax, Related mildly avid linear subpleural



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أ.د. ياسر عبد العظيم عباس
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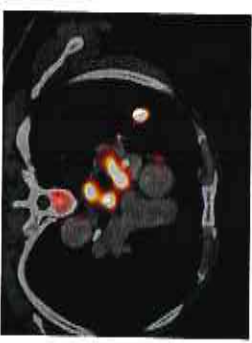
PATIENT ID: 1516881
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fibrotic sheet seen at a superior level at right apex, with hyperdense calcific foci, achieving up to 3.2 SUVmax.

- Mild partially encysted right pleural effusion with irregular zones of hypermetabolic right pleural nodular thickening seen mainly implicating the lateral, posterior costal, and to lesser extent diaphragmatic pleural surface as well as the right costophrenic recess. They reach a thickness of ± 13 mm and achieve up to 5.6 SUVmax opposite the lateral costal pleural surface.

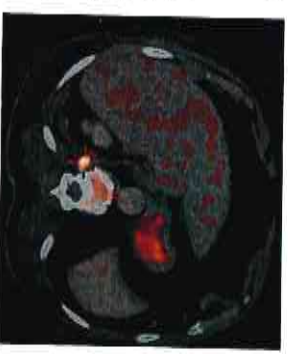


- Small ill-defined right lower lobar metabolically active nodular lung lesion is seen at its posterior segment, measuring 11 mm and achieving up to 7.04 SUVmax.



- Multiple enlarged hypermetabolic partially amalgamated mediastinal (pre-caval, right internal mammary, pre-tracheal/retro-caval, right tracheobronchial, carinal, sub-carinal, para-esophageal, right cardio-phrenic), and right hilar lymph nodes are seen. The most active one currently seen at the right tracheobronchial/hilar group achieves up to 11.4 SUVmax, while the largest one is seen at the sub-carinal/para-esophageal group measures 29 x 40 mm). Such nodal lesions show rounded outline with central hypodense zone of necrosis seen in some of them and few related foci of dystrophic calcification.

- Few hypermetabolic right lower paravertebral nodular lesions are seen partially abutting the related right lower posterior costal pleural surface. The most active one currently seen measuring 11 mm and achieves up to 7.26 SUVmax, likely nodal in nature.



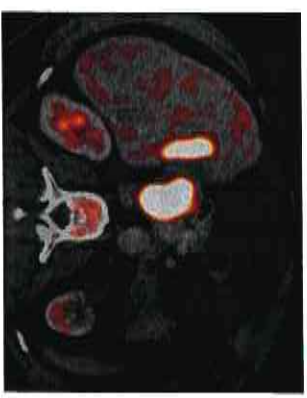
- No suspicious hypermetabolic left-sided pulmonary nodules or masses detected.
- No left pleural effusion or thickening detected.
- Normal tracer biodistribution uptake is noted in the myocardium.

MR. ABDALLAH SAYED IBRAHIM
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PATIENT ID: 1516881
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C] Abdomen & Pelvis:-

- Left adrenal isodense hypermetabolic nodular lesion is seen measuring 22.5 x 15mm and achieving up to 5.59 SUVmax.
- **The liver** is mildly enlarged in size, showing hypermetabolic focal lesion subcapsular at segment V adherent to the gallbladder fossa, measuring $\pm 43.5 \times 27$ mm and achieves up to 12.25 SUVmax.
- Enlarged hypermetabolic upper abdominal lymph nodes seen implicating the porta hepatitis, pre-caval/peri-pancreatic, and aorto-caval groups. The largest and most active one is seen at the porta hepatitis/pre-caval group measuring $\pm 43 \times 23$ mm and achieving up to 13.33 SUVmax.
- Multiple small non-complicated colonic diverticulosis are noted with clear surrounding fat planes.
- Mildly exaggerated physiological FDG uptake along different colonic bowel loops.
- No free ascites.
- Physiological uptake in the right adrenal gland, spleen and pancreas.
- Physiological excretion of FDG in the kidneys. Large right renal calyceal dense stone is seen measuring 19 mm, with no significant back pressure changes detected.
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.



D] Musculoskeletal:-

- No metabolically active osseous lesions detected.
- Diffuse hyperactive bone marrow likely attributed to element of cytopenia for laboratory correlation.
- Mild diffuse spondylodegenerative changes of the examined spine.

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PATIENT ID: 1516881

PROF. DR. HANY HASSAN

STUDY DATE: 27/06/2026

CONCLUSION:-

- Few hypermetabolic right-sided nodular lung lesions with irregular hypermetabolic right pleural nodular lesions and multiple hypermetabolic supra, infra-diaphragmatic lymphadenopathies, together with hypermetabolic right lobe hepatic focal lesion and left adrenal lesion, likely metastatic deposits, for follow-up.

MUCH OBLIGED
PROF.DR. YASSER IBRAHIM
PROF DR. OMAR HUSSEIN

A. PROF. DR. BASANT RAIEF

10/26/2024 4:30:39PM

Patient name	عبدالله سيد ابراهيم حسين			
Patient ID	Ref By Prof.Dr. Ahmed Tarek Hussien Kamel		File number	
 U2405027792	Age 61 Year Sex Male		 IPC240009333	
Lab No 2561149	Collection date 20/10/2024 20:36	Report date	Room No 4023	Location قسم الداخلي

pathology

Ref No. : 3 848

Referred by Prof.

HANY HASSAN

Clinical Presentation :

RIGHT UPPER LOBE MASS

Gross :

FOUR DESIGNATED CONTAINERS WERE RECEIVED:

- 1) Right upper lobectomy measured 13x12x3cm, showed an ill defined greyish white firm mass, measured 1x1cm, located at 5cm from surgical margins. Rest of lung tissue was congested.
- 2) Lymph node (10), submitted as 5: Three pieces, collectively measured 1.5x1cm, totally submitted.
- 3) R lymph node (4), submitted as 6: one piece measured 2x1.5cm.
- 4) R Lymph node (2), submitted as 7: Three pieces, collectively measured 2x2cm, totally submitted.

Microscopic :

Sections examined from the specimen (1) received revealed lung tissue with ulceration and emphysematous change showing infiltrating tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear pleomorphism and hyperchromasia, Arranged in distorted and fused acini, mucosa secreting glands. There is moderate desmoplasia and focal necrosis.

There is moderate anthracosis.

Lympho-vascular space invasion present.

Free pleura.

Free hilar surgical margins.

Adherent hilar lymph node (0/1).

Sections examined from the specimen (2) received revealed lymph nodes showing reactive hyperplastic changes with no metastatic deposits in 5 lymph nodes.

Sections examined from the specimen (3) received revealed lymph nodes revealed metastatic deposits in 2 out of 6 lymph nodes.

Sections examined from the specimen (4) received revealed lymph nodes showing reactive hyperplastic changes with no metastatic deposits in 3 lymph nodes.

Diagnosis :

- 1] RIGHT UPPER LOBECTOMY: INFILTRATING ADENOCARCINOMA GRADE II, FREE HILAR SURGICAL MARGINS, NO HILAR NODAL METASTASIS, L.N 0/1.
 - 2] LYMPH NODE (10), SUBMITTED AS 5: NO NODAL METASTASIS, L.N 0/5.
 - 3] R LYMPH NODE (4), SUBMITTED AS 6: POSITIVE LYMPH NODE METASTASIS, L.N 2/6.
 - 4] LYMPH NODE (2), SUBMITTED AS 7: NO NODAL METASTASIS, L.N 0/3.
- * STAGE : pT1a N1.

Patient's Name: Abdallah Said Ibrahim
Date: 2026-07-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE CT EXAMINATION OF THE CHEST WITHOUT
CONTRAST REVEALED:-**

- Mild right pleural effusion with air pockets seen. It is seen associated with right lower lobe subsegmental lung collapse/ consolidation.
- Few scattered right upper and lower lobe pulmonary nodules largest measuring 27 x 17 mm at the right middle lobe with spiculated margins.
- Multiple scattered upper and lower lobe atelectatic bands.
- Multiple mediastinal lymph nodes largest measuring 37 x 28 mm at the subcarinal region. Some of them being calcified.
- No pericardial effusion.
- Normal CT appearance of the heart and great vessels.
- Upper abdominal cuts revealed right renal stone and bulky left adrenal gland.

Opinion:-

- ❖ Mild right hydro-pneumothorax with underlying right lower lobe subsegmental lung collapse/ consolidation.
- ❖ Few right upper and lower lobe suspicious pulmonary nodules for contrast enhanced study.
- ❖ Mediastinal lymphadenopathy.
- ❖ Right renal stone and bulky left adrenal gland for dedicated study of the abdomen.

Dr.sherif Wagdy